Nursing Essentials

Priority: Assess bleeding and uterine tone together with vital signs, pain, bladder function, mental health, and the client's concerns.

FOCUS 1

Assess

- **Fundus and lochia:** evaluate tone, position, amount, odor, clots, and the trend over time.
- **Bladder and comfort:** retention can displace the uterus and impair contraction; reassess after voiding or ordered intervention.
- **Other checks:** incision / perineum, breasts, extremities, bowel function, mobility, and signs of infection.

FOCUS 2

Escalate

- **Hemorrhage:** increasing bleeding, large clots, dizziness, syncope, tachycardia, or poor perfusion.
- **Cardiopulmonary / neurologic:** chest pain, dyspnea, severe headache, vision changes, or seizure.
- **Infection / thrombosis:** fever, worsening focal pain, foul discharge, or unilateral leg swelling and pain.

FOCUS 3

Support

- **Feeding:** assess latch, milk transfer, breast comfort, or formula needs without judgment.
- **Daily needs:** hydration, nutrition, sleep support, safe mobility, pain management, and medication review.
- **Teaching:** hygiene, wound care, contraception, follow-up, and when / how to seek help.

Mental health belongs in the assessment

Make it stick: Do not use Homans sign to screen for DVT. Evaluate concerning symptoms and arrange the appropriate diagnostic assessment.

Focus	What to know	Nursing application
Ask directly	mood, anxiety, coping, safety, support, and intrusive thoughts or distress.	Ask about safety
Escalate immediately	suicidal intent, thoughts of harming the infant, psychosis, or inability to maintain safety.	Arrange urgent help
Follow through	connect the client with timely assessment and support; do not dismiss persistent symptoms as normal adjustment.	Connect ongoing support

RED FLAGS · ESCALATE PROMPTLY

Heavy bleeding, syncope, chest pain, dyspnea, severe headache, seizure, suicidal intent, or psychosis requires immediate help.